

Value of Information Analysis

METIMMOX-1 Economic Evaluation

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1 Overview

This report presents the results of an Expected Value of Partially Perfect Information (EVPPI) analysis for the METIMMOX-1 decision-analytic model. The analysis evaluates the value of reducing uncertainty in individual model parameters across multiple scenarios with varying willingness-to-pay (WTP) thresholds and nivolumab drug costs.

Key Feature: This report compares EVPI and EVPPI results across three different model structures (Model A, B, C) to assess the robustness of value of information findings to modeling assumptions about how biomarker interactions are specified.

The Expected Value of Partially Perfect Information (EVPPI) quantifies the value of eliminating uncertainty in specific model parameters. It represents the maximum amount that should be paid for additional research to perfectly inform a particular parameter. EVPI (Expected Value of Perfect Information) represents the value of eliminating all uncertainty, and EVPPI for any parameter cannot exceed total EVPI.

1.1 Population-Level Context

To inform research prioritization decisions, EVPPI values are scaled from per-patient estimates to the Norwegian eligible population:

- **Annual incidence:** 1500 MSS/pMMR mCRC patients per year
- **Research horizon:** 10 years
- **Discount rate:** 3.5% for future research benefits
- **Effective population size:** 12475 discounted patient-years

Population-level EVPPI represents the total expected value (in millions of EUR) of eliminating uncertainty about a parameter or parameter group across the entire eligible population over the research horizon. This scaling accounts for the discounted flow of future patients who would benefit from the research.

Data sources: The annual incidence estimate is based on Norwegian Cancer Registry data (approximately 4,000 annual CRC cases) and international literature estimates for MSS/pMMR proportion (70-75% of metastatic cases) and metastatic presentation rates (approximately 50%), yielding a conservative estimate of 1,500 eligible patients per year.

1.2 Treatment-Biomarker Interaction Parameters

In addition to cost, utility, and prevalence parameters, this analysis evaluates the EVPPI for treatment-biomarker interaction coefficients extracted from the survival models. These parameters (prefixed **b_**) represent the effect modification of experimental treatment by biomarker status – the core clinical question underlying biomarker-guided therapy.

The interaction coefficients are extracted from the same resampled survival models used in the PSA, ensuring alignment between coefficient variation and outcome variation. For each biomarker strategy, the interaction coefficient is extracted from that strategy’s own resampled

model (e.g., `b_crp_rx_os` from `sampling_models$crp`). Both OS and PFS interaction coefficients are analyzed individually and as per-biomarker groups.

High EVPPI for interaction parameters would indicate that better estimates of differential treatment benefit by biomarker status would significantly reduce decision uncertainty – directly informing future trial design and biomarker validation studies.

2 Model Structure Definitions

We compare three model structures to assess the robustness of value of information results to the specification of biomarker-treatment interactions in the survival models.

Table 1: Model structure definitions used in the analysis

Model	Name	Formula Pattern	Description
Model A	Joint	Age + sex + Rx + crp*Rx + tlr*Rx + tmb_braf*Rx	All biomarkers and all treatment interactions in one unified model
Model B	Focused	Age + sex + Rx + crp + tmb_braf + crp:Rx + tmb_braf:Rx	Per-strategy formulas: CRP/TMB_BRAF with both interactions, TLR with TLR-only interaction
Model C	Separate	Age + sex + Rx + [biomarker]:Rx	Only one biomarker and its treatment interaction per model (simplest specification)

Rationale: Model A (Joint) is the most comprehensive specification but may suffer from over-fitting with limited sample size. Model C (Separate) is the simplest but ignores potential confounding from other biomarkers. Model B (Focused) uses a shared formula with CRP and TMB/BRAF (excluding TLR), providing simultaneous estimates for both interactions while adjusting for each other. Comparing results across all three structures provides insight into the structural uncertainty of our findings.

3 Scenario Definitions

Table 2: Analysis scenario definitions. Higher WTP thresholds (EUR100,000 and EUR150,000) are arbitrary exploratory thresholds, though the highest severity adjustment in Norwegian guidelines may fall between these values. The decreased nivolumab cost (EUR4,641) represents an 80% discount compared to the 40% discount in the base case scenario.

Scenario Name	WTP Threshold	Nivolumab Cost
Base Case	EUR 51,000	EUR13,923
Decreased Nivolumab Cost	EUR 51,000	EUR 4,641
WTP €100,000	EUR100,000	EUR13,923
WTP €100,000 + Decreased Nivolumab	EUR100,000	EUR 4,641
WTP €150,000	EUR150,000	EUR13,923
WTP €150,000 + Decreased Nivolumab	EUR150,000	EUR 4,641

4 Base Case Scenario

Willingness-to-Pay Threshold: EUR51,000

Nivolumab Cost: EUR13,923

4.1 EVPI Comparison Across Model Structures

Table 3: Expected Value of Perfect Information (EVPI) by model structure - Base Case

	Model	EVPI
5	Model A: Joint	EUR258.97
56	Model B: Focused	EUR307.05
57	Model C: Separate	EUR153.69

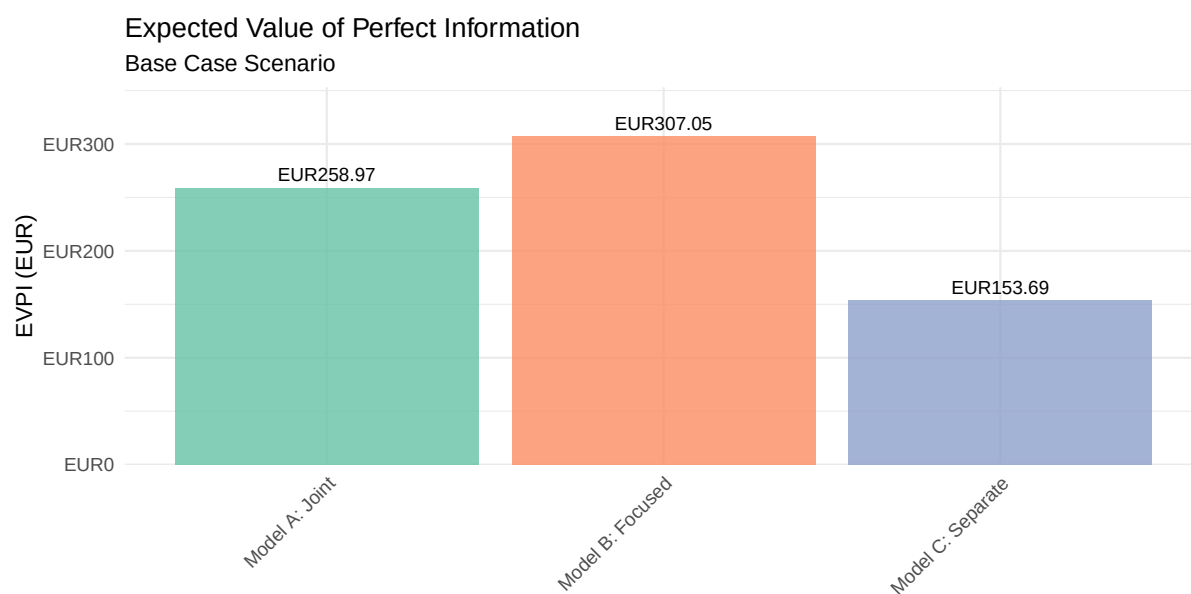


Figure 1: EVPI comparison across model structures (Base Case)

Table 4: Per-patient EVPPI results by model structure - Base Case (ranked by average importance)

Parameter	EVPPI (A)	% (A)	EVPPI (B)	% (B)	EVPPI (C)	% (C)
u_np	EUR192.01	74.1%	EUR159.95	52.1%	EUR153.69	100%
c_other_last	EUR115.87	44.7%	EUR0	0%	EUR91.48	59.5%
c_other_follow	EUR46.7	18%	EUR25.08	8.2%	EUR65.99	42.9%
c_test_CT	EUR64.42	24.9%	EUR0	0%	EUR0	0%
c_other_baseline	EUR64.2	24.8%	EUR0	0%	EUR0	0%
c_drug_FLOX	EUR2.05	0.8%	EUR39.59	12.9%	EUR16.42	10.7%
u_p	EUR46.99	18.1%	EUR3.62	1.2%	EUR0	0%
c_test_blood	EUR44.02	17%	EUR0	0%	EUR1.92	1.3%
c_test_NGS	EUR0	0%	EUR21.78	7.1%	EUR4	2.6%
c_other_visit	EUR15.99	6.2%	EUR0	0%	EUR0	0%
c_drug_nivo	EUR0	0%	EUR0	0%	EUR0	0%

Table 5: Population-level EVPPI results by model structure - Base Case (millions of EUR over 10 years, discounted at 3.5%, ranked by average importance)

Parameter	Pop. EVPPI (A)	% (A)	Pop. EVPPI (B)	% (B)	Pop. EVPPI (C)	% (C)
u_np	EUR2.40M	74.1%	EUR2.00M	52.1%	EUR1.92M	100%
c_other_last	EUR1.45M	44.7%	EUR0.00M	0%	EUR1.14M	59.5%
c_other_follow	EUR0.58M	18%	EUR0.31M	8.2%	EUR0.82M	42.9%
c_test_CT	EUR0.80M	24.9%	EUR0.00M	0%	EUR0.00M	0%
c_other_baseline	EUR0.80M	24.8%	EUR0.00M	0%	EUR0.00M	0%
c_drug_FLOX	EUR0.03M	0.8%	EUR0.49M	12.9%	EUR0.20M	10.7%
u_p	EUR0.59M	18.1%	EUR0.05M	1.2%	EUR0.00M	0%
c_test_blood	EUR0.55M	17%	EUR0.00M	0%	EUR0.02M	1.3%
c_test_NGS	EUR0.00M	0%	EUR0.27M	7.1%	EUR0.05M	2.6%
c_other_visit	EUR0.20M	6.2%	EUR0.00M	0%	EUR0.00M	0%
c_drug_nivo	EUR0.00M	0%	EUR0.00M	0%	EUR0.00M	0%

4.2 EVPPI Results by Model Structure

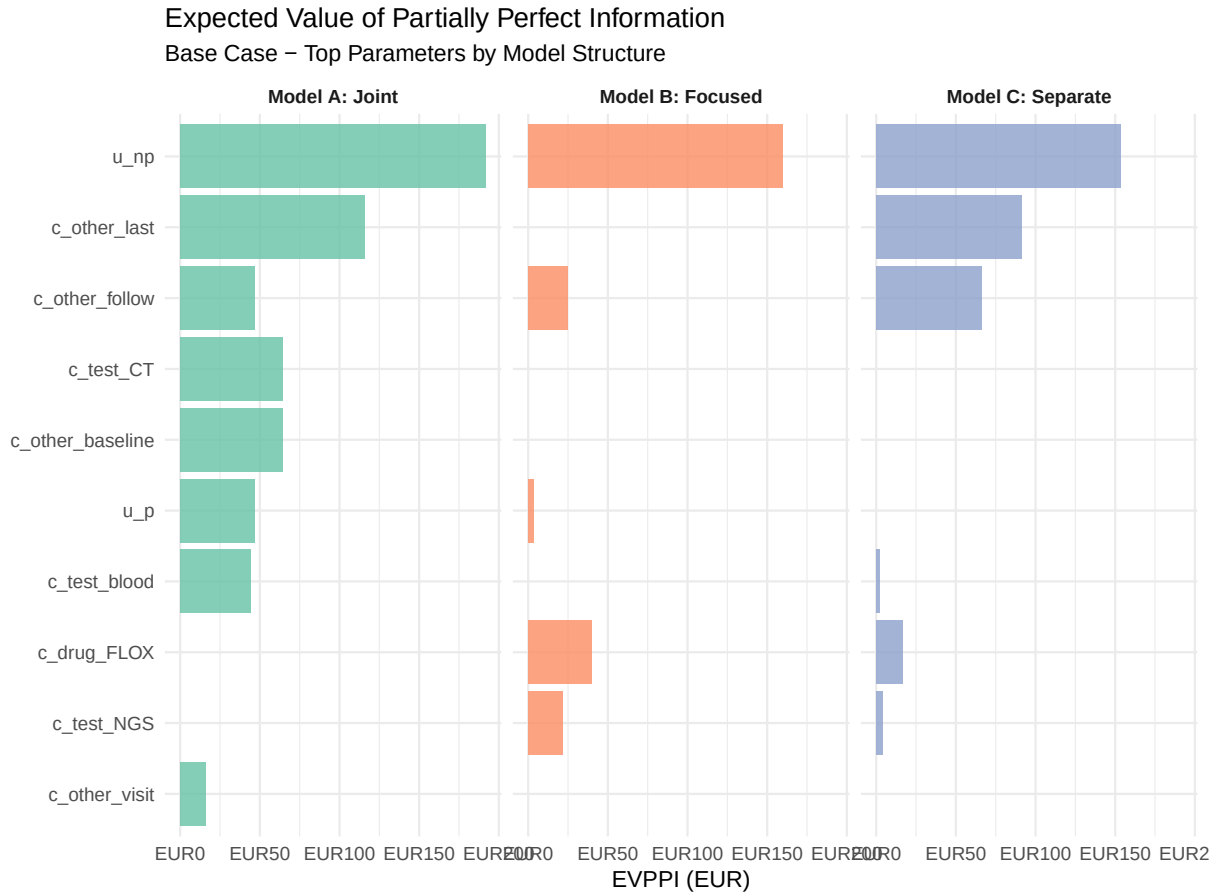


Figure 2: EVPPI by parameter and model structure (Base Case)

4.3 Interpretation

4.3.1 Population-Level Research Value

Per-Patient Level: Utility parameters (u_np, u_p) show the highest per-patient EVPPI, ranging from approximately EUR800-1,100 in the base case, while cost parameters show lower EVPPI (typically <EUR200), reflecting less uncertainty in cost estimates.

Population Level: Scaling to the Norwegian eligible population (1500 patients/year over 10 years, discounted at 3.5%), utility parameter uncertainty translates to **EUR2.4 million** in potential research value. This population-level value provides context for research investment decisions. For example, a study to reduce uncertainty in utility values (e.g., a large-scale quality-of-life measurement study in this patient population) could be justified if its cost is substantially less than EUR2.4 million.

The positive EVPI indicates decision uncertainty at the base case WTP threshold. This suggests that additional research to reduce parameter uncertainty could have value. The comparison across model structures shows whether this finding is robust to the specification of biomarker-treatment interactions.

5 Scenario 1: Decreased Nivolumab Cost

Willingness-to-Pay Threshold: EUR51,000

Nivolumab Cost: EUR4,641 (80% discount)

5.1 EVPI Comparison Across Model Structures

Table 6: Expected Value of Perfect Information (EVPI) by model structure - Scenario 1

	Model	EVPI
53	Model A: Joint	EUR3667.66
531	Model B: Focused	EUR4324.15
532	Model C: Separate	EUR4161.04

5.2 EVPPI Results by Model Structure

Table 7: EVPPI results by model structure - Scenario 1: Decreased Nivolumab Cost

Parameter	EVPPI (A)	% (A)	EVPPI (B)	% (B)	EVPPI (C)	% (C)
u_np	EUR158.21	4.3%	EUR237.18	5.5%	EUR232.83	5.6%
c_drug_FLOX	EUR68.52	1.9%	EUR139.66	3.2%	EUR29.09	0.7%
c_other_last	EUR18.15	0.5%	EUR66.73	1.5%	EUR106.43	2.6%
c_other_follow	EUR0	0%	EUR33.01	0.8%	EUR72.02	1.7%
c_other_baseline	EUR62.13	1.7%	EUR4.05	0.1%	EUR36.8	0.9%
c_test_blood	EUR86.05	2.3%	EUR12.45	0.3%	EUR0	0%
c_test_NGS	EUR88.65	2.4%	EUR0	0%	EUR0	0%
u_p	EUR40.13	1.1%	EUR0	0%	EUR47.39	1.1%
c_test_CT	EUR40.8	1.1%	EUR0	0%	EUR12.94	0.3%
c_drug_nivo	EUR13.97	0.4%	EUR16.86	0.4%	EUR19.15	0.5%
c_other_visit	EUR0	0%	EUR3.49	0.1%	EUR43.94	1.1%

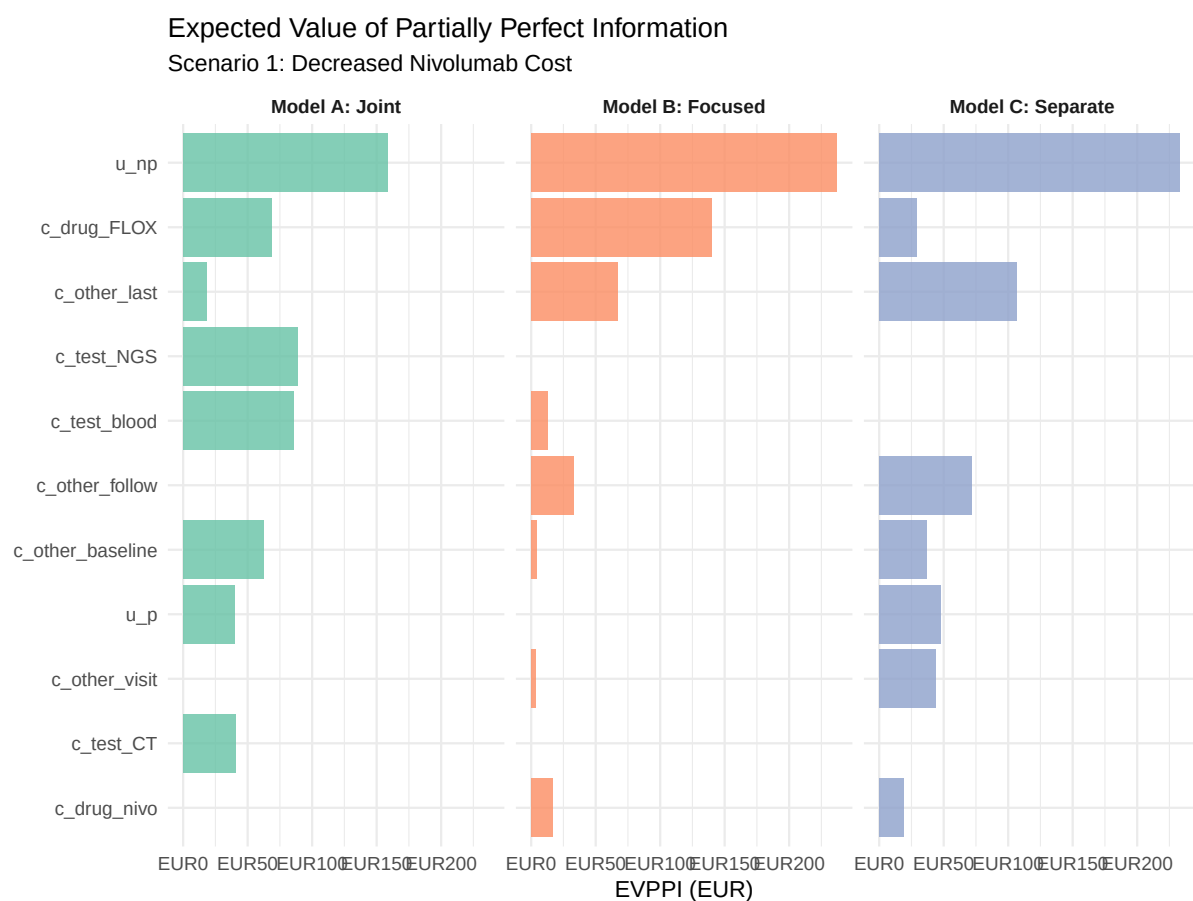


Figure 3: EVPPI by parameter and model structure (Scenario 1)

6 Scenario 2: Higher WTP Threshold (EUR100,000)

Willingness-to-Pay Threshold: EUR1e+05

Nivolumab Cost: EUR13,923 (standard)

6.1 EVPI Comparison Across Model Structures

Table 8: Expected Value of Perfect Information (EVPI) by model structure - Scenario 2

	Model	EVPI
51	Model A: Joint	EUR3549.34
511	Model B: Focused	EUR4488.2
512	Model C: Separate	EUR3911.61

6.2 EVPPI Results by Model Structure

Table 9: EVPPI results by model structure - Scenario 2: WTP EUR100,000

Parameter	EVPPI (A)	% (A)	EVPPI (B)	% (B)	EVPPI (C)	% (C)
u_np	EUR388.54	10.9%	EUR322.38	7.2%	EUR399.62	10.2%
c_other_last	EUR165.73	4.7%	EUR0	0%	EUR108.83	2.8%
c_other_follow	EUR97.08	2.7%	EUR41.3	0.9%	EUR116.59	3%
c_other_baseline	EUR117.26	3.3%	EUR0	0%	EUR0	0%
c_test_CT	EUR114.04	3.2%	EUR0	0%	EUR0	0%
u_p	EUR85.64	2.4%	EUR2.83	0.1%	EUR0	0%
c_test_blood	EUR71.39	2%	EUR0	0%	EUR0	0%
c_other_visit	EUR44.22	1.2%	EUR0	0%	EUR0	0%
c_test_NGS	EUR0	0%	EUR39.52	0.9%	EUR2.29	0.1%
c_drug_FLOX	EUR0	0%	EUR27.62	0.6%	EUR0	0%
c_drug_nivo	EUR0	0%	EUR0	0%	EUR0	0%

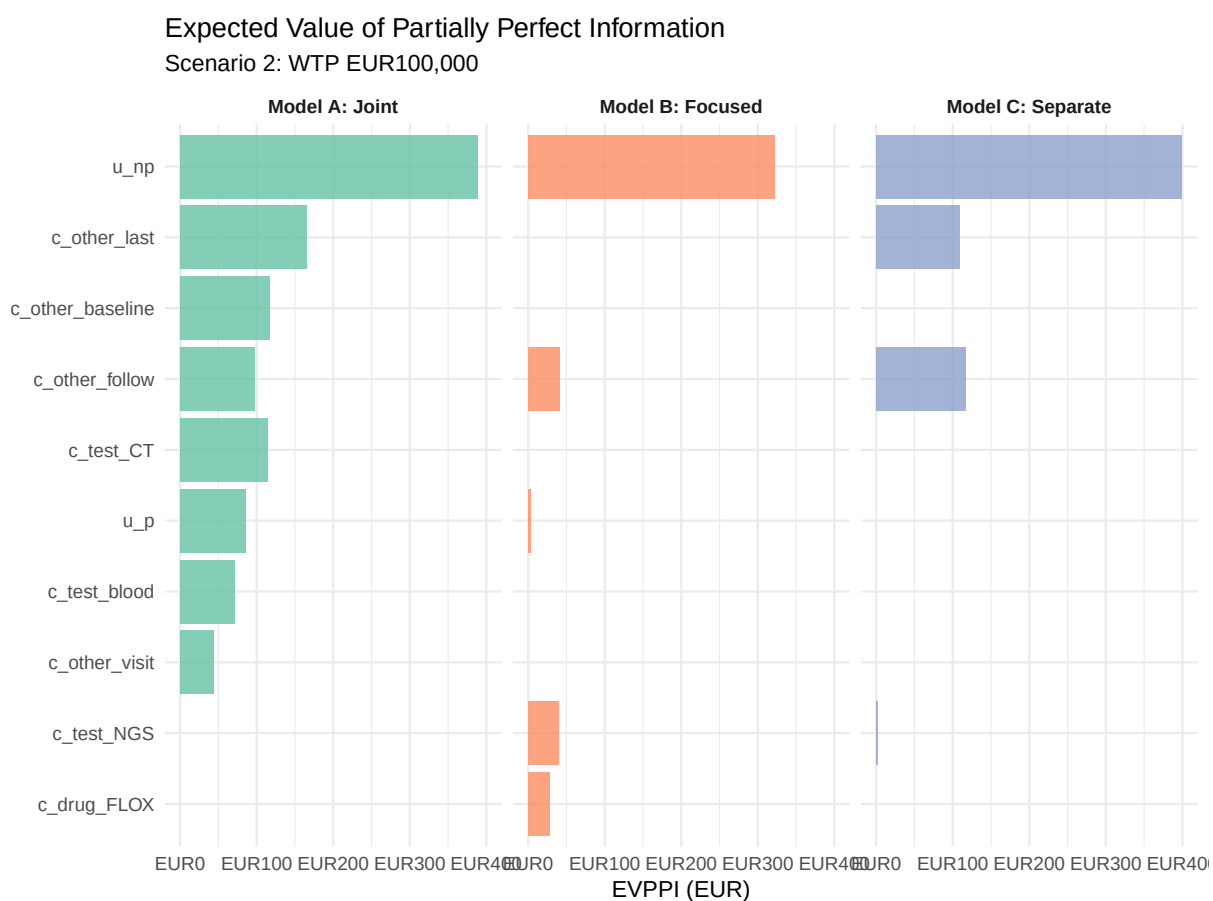


Figure 4: EVVPI by parameter and model structure (Scenario 2)

7 Scenario 3: Higher WTP with Decreased Nivolumab Cost

Willingness-to-Pay Threshold: EUR1e+05

Nivolumab Cost: EUR4,641 (80% discount)

7.1 EVPI Comparison Across Model Structures

Table 10: Expected Value of Perfect Information (EVPI) by model structure - Scenario 3

	Model	EVPI
54	Model A: Joint	EUR13672.92
541	Model B: Focused	EUR14730.07
542	Model C: Separate	EUR15998.1

7.2 EVPPI Results by Model Structure

Table 11: EVPPI results by model structure - Scenario 3: WTP EUR100,000 + Decreased Nivolumab

Parameter	EVPPI (A)	% (A)	EVPPI (B)	% (B)	EVPPI (C)	% (C)
u_np	EUR319.41	2.3%	EUR1251.48	8.5%	EUR1196.94	7.5%
c_drug_nivo	EUR301.33	2.2%	EUR844.91	5.7%	EUR260.16	1.6%
u_p	EUR85.03	0.6%	EUR433.08	2.9%	EUR125.89	0.8%
c_test_NGS	EUR154.93	1.1%	EUR403.55	2.7%	EUR0	0%
c_drug_FLOX	EUR103.44	0.8%	EUR284.36	1.9%	EUR18.54	0.1%
c_other_last	EUR0	0%	EUR208.67	1.4%	EUR156.4	1%
c_other_visit	EUR0	0%	EUR261.56	1.8%	EUR72.51	0.5%
c_other_baseline	EUR121.26	0.9%	EUR119.28	0.8%	EUR86.74	0.5%
c_test_blood	EUR156.13	1.1%	EUR129.46	0.9%	EUR0	0%
c_other_follow	EUR0	0%	EUR91.82	0.6%	EUR151.06	0.9%
c_test_CT	EUR75.65	0.6%	EUR0	0%	EUR14.78	0.1%

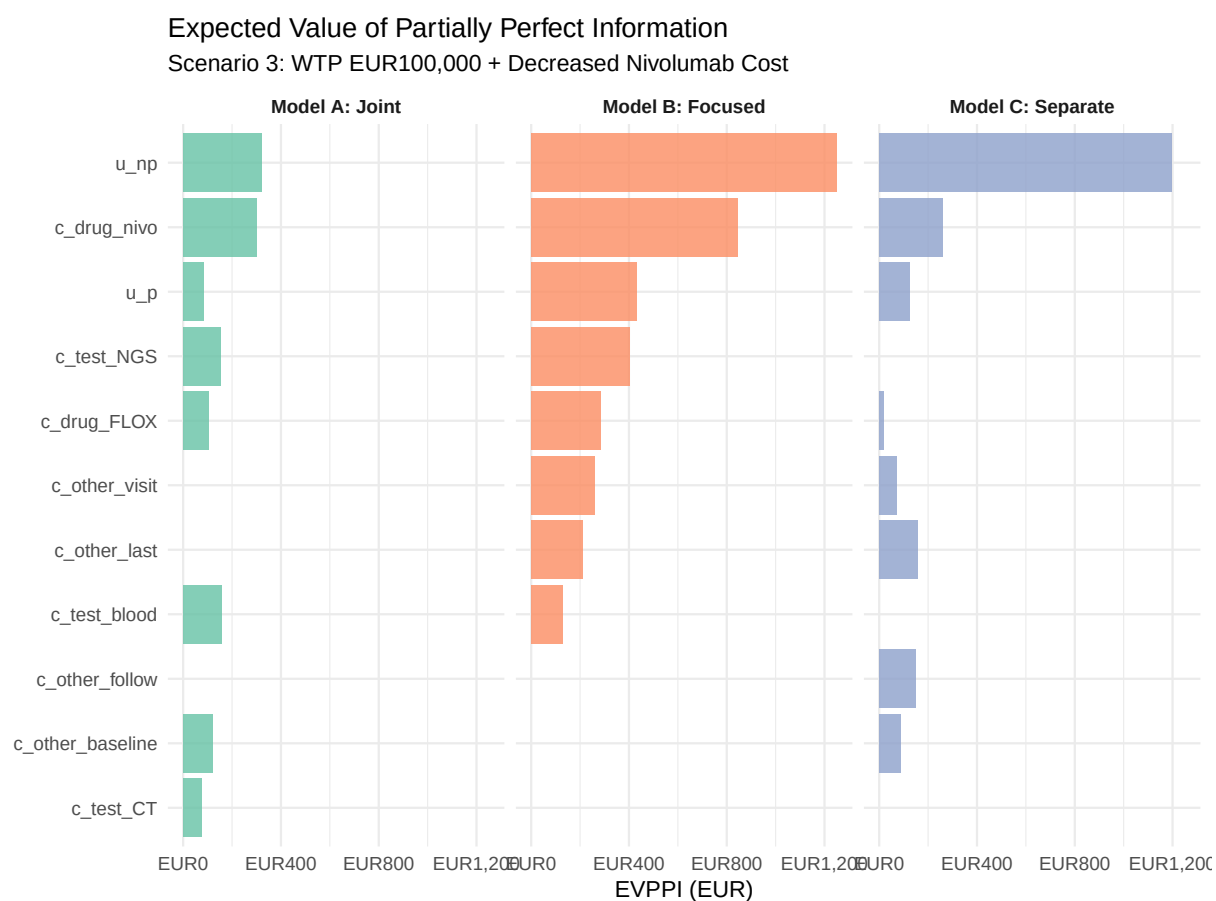


Figure 5: EVPPPI by parameter and model structure (Scenario 3)

8 Scenario 4: Highest WTP Threshold (EUR150,000)

Willingness-to-Pay Threshold: EUR150,000

Nivolumab Cost: EUR13,923 (standard)

8.1 EVPI Comparison Across Model Structures

Table 12: Expected Value of Perfect Information (EVPI) by model structure - Scenario 4

Model		EVPI
52	Model A: Joint	EUR10833.54
521	Model B: Focused	EUR12886.78
522	Model C: Separate	EUR12556.28

8.2 EVPPI Results by Model Structure

Table 13: EVPPI results by model structure - Scenario 4: WTP EUR150,000

Parameter	EVPPI (A)	% (A)	EVPPI (B)	% (B)	EVPPI (C)	% (C)
u_np	EUR589.08	5.4%	EUR488.12	3.8%	EUR602.23	4.8%
c_other_follow	EUR148.48	1.4%	EUR57.86	0.4%	EUR168.22	1.3%
c_other_last	EUR216.61	2%	EUR0	0%	EUR126.52	1%
c_other_baseline	EUR171.41	1.6%	EUR0	0%	EUR0	0%
c_test_CT	EUR164.66	1.5%	EUR0	0%	EUR0	0%
u_p	EUR125.08	1.2%	EUR2.03	0%	EUR0	0%
c_test_blood	EUR99.32	0.9%	EUR0	0%	EUR0	0%
c_other_visit	EUR73.02	0.7%	EUR0	0%	EUR0	0%
c_test_NGS	EUR0	0%	EUR57.62	0.4%	EUR0.55	0%
c_drug_FLOX	EUR0	0%	EUR15.41	0.1%	EUR0	0%
c_drug_nivo	EUR0	0%	EUR0	0%	EUR0	0%

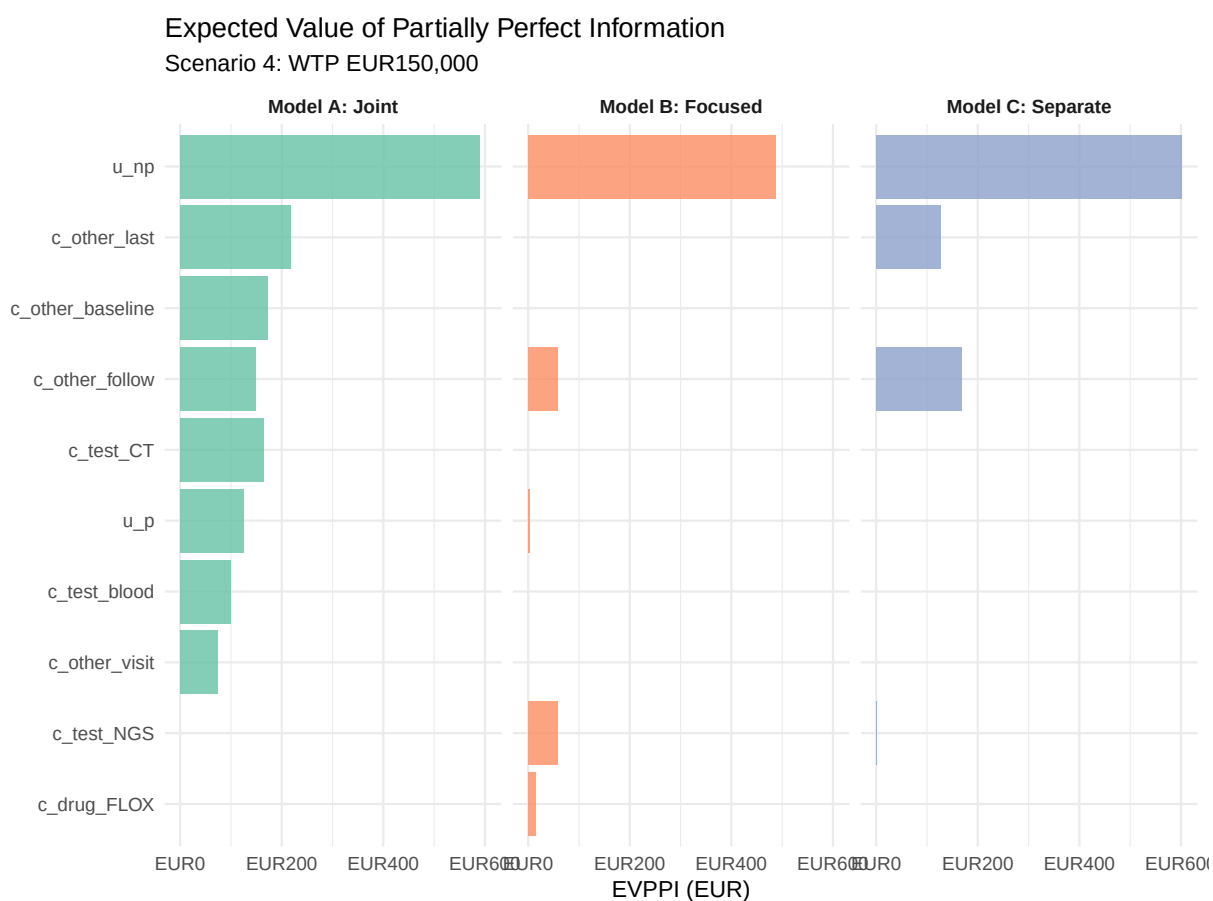


Figure 6: EVPPI by parameter and model structure (Scenario 4)

9 Scenario 5: Highest WTP with Decreased Nivolumab Cost

Willingness-to-Pay Threshold: EUR150,000

Nivolumab Cost: EUR4,641 (80% discount)

9.1 EVPI Comparison Across Model Structures

Table 14: Expected Value of Perfect Information (EVPI) by model structure - Scenario 5

	Model	EVPI
55	Model A: Joint	EUR22346.86
551	Model B: Focused	EUR21015.48
552	Model C: Separate	EUR23778.67

9.2 EVPPI Results by Model Structure

Table 15: EVPPI results by model structure - Scenario 5: WTP EUR150,000 + Decreased Nivolumab

Parameter	EVPPI (A)	% (A)	EVPPI (B)	% (B)	EVPPI (C)	% (C)
u_np	EUR930.48	4.2%	EUR590.73	2.8%	EUR796.32	3.3%
c_drug_nivo	EUR406.38	1.8%	EUR186.4	0.9%	EUR238.97	1%
c_test_NGS	EUR174.85	0.8%	EUR287.17	1.4%	EUR0	0%
u_p	EUR85.77	0.4%	EUR49.31	0.2%	EUR265.88	1.1%
c_drug_FLOX	EUR193.5	0.9%	EUR177.38	0.8%	EUR0	0%
c_test_CT	EUR131.76	0.6%	EUR0	0%	EUR172.3	0.7%
c_other_follow	EUR0	0%	EUR56.82	0.3%	EUR221.16	0.9%
c_test_blood	EUR0	0%	EUR70.55	0.3%	EUR204.88	0.9%
c_other_baseline	EUR60.32	0.3%	EUR50.33	0.2%	EUR90.51	0.4%
c_other_visit	EUR0	0%	EUR0	0%	EUR126.35	0.5%
c_other_last	EUR0	0%	EUR5.06	0%	EUR6.04	0%

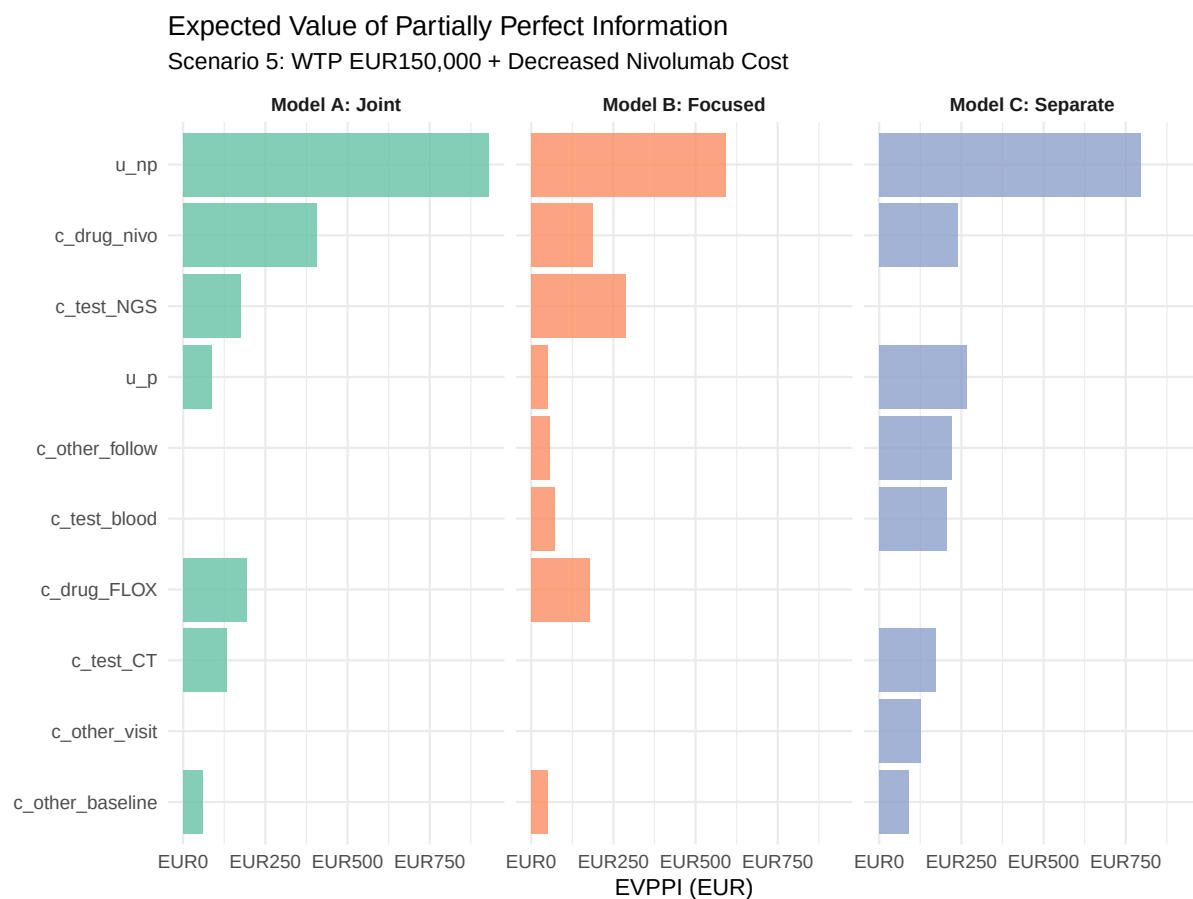


Figure 7: EVPPPI by parameter and model structure (Scenario 5)

10 Comparative Analysis Across Scenarios and Models

10.1 EVPI Comparison

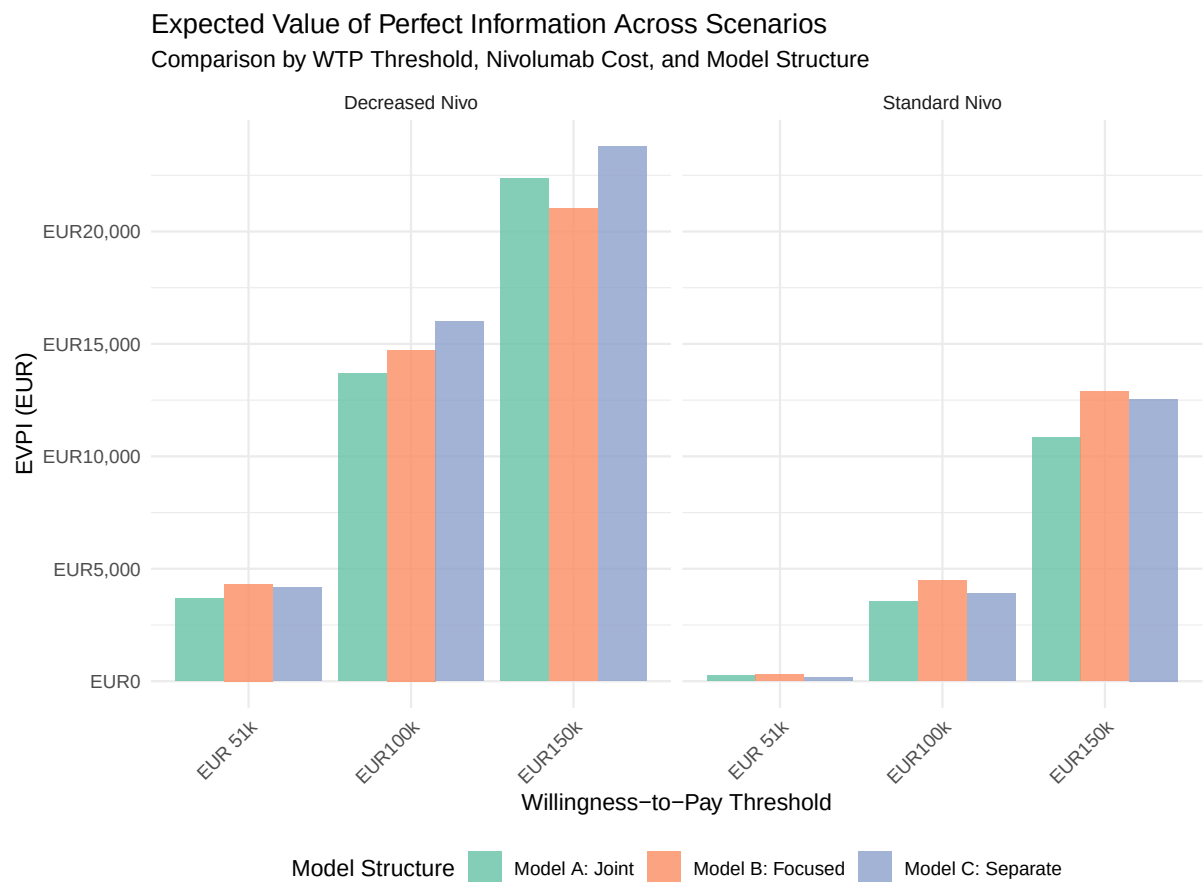


Figure 8: Expected Value of Perfect Information across scenarios and model structures

10.2 EVPPI Parameter Heatmap by Model Structure

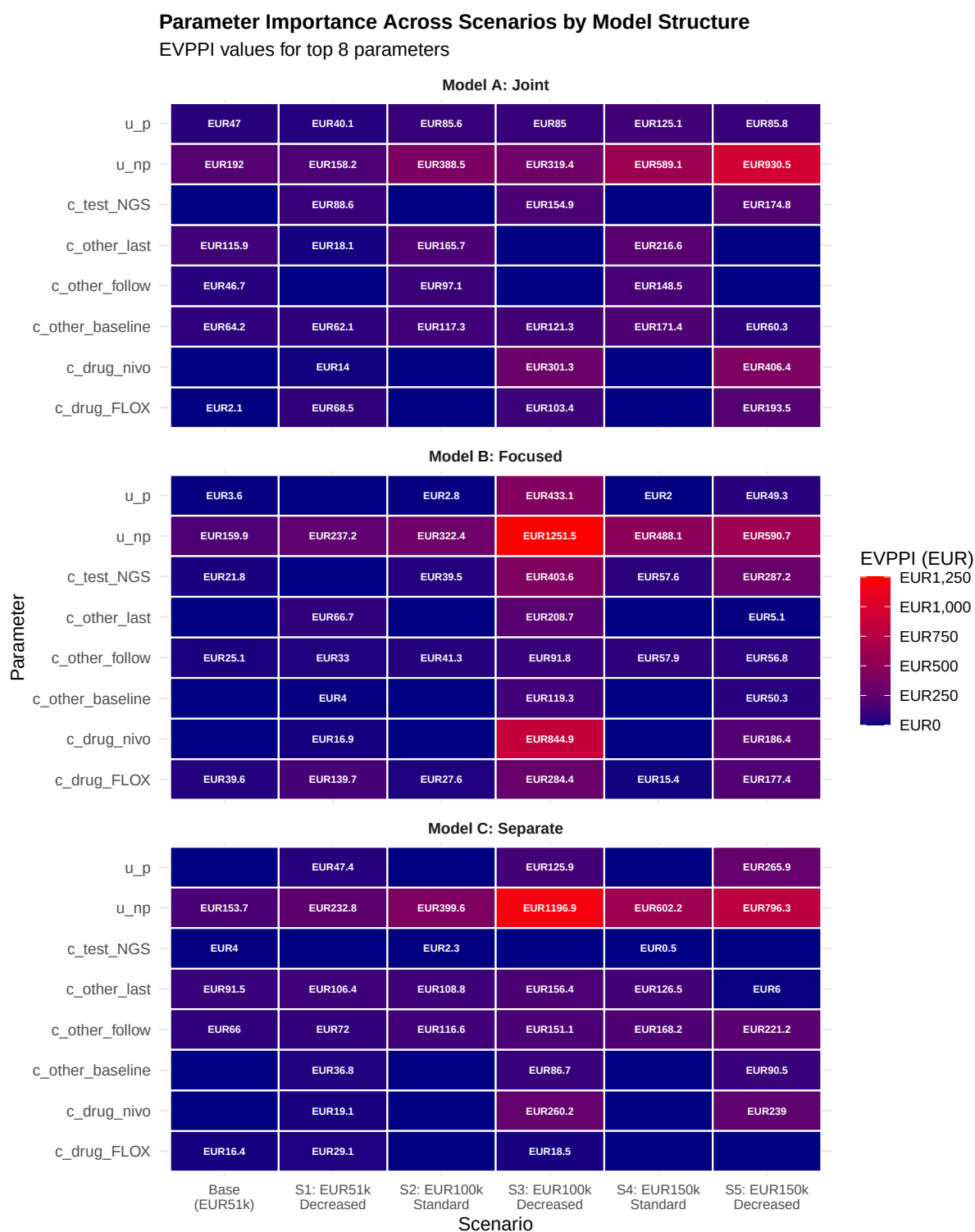


Figure 9: Parameter importance across scenarios by model structure (top 8 parameters)

Table 16: Summary of EVPI and top EVPPI by model structure and scenario

Model	Scenario	WTP	EVPI	Top Parameter	Max EVPPI	% of EVPI
Model A: Joint	Base Case	EUR 51,000	EUR258.97	u_np	EUR192.01	74.1
Model A: Joint	Decreased Nivolumab Cost	EUR 51,000	EUR3667.66	u_np	EUR158.21	4.3
Model A: Joint	WTP €100,000	EUR100,000	EUR3549.34	u_np	EUR388.54	10.9
Model A: Joint	WTP €100,000 + Decreased Nivolumab	EUR100,000	EUR13672.92	u_np	EUR319.41	2.3
Model A: Joint	WTP €150,000	EUR150,000	EUR10833.54	u_np	EUR589.08	5.4
Model A: Joint	WTP €150,000 + Decreased Nivolumab	EUR150,000	EUR22346.86	u_np	EUR930.48	4.2
Model B: Focused	Base Case	EUR 51,000	EUR307.05	u_np	EUR159.95	52.1
Model B: Focused	Decreased Nivolumab Cost	EUR 51,000	EUR4324.15	u_np	EUR237.18	5.5
Model B: Focused	WTP €100,000	EUR100,000	EUR4488.2	u_np	EUR322.38	7.2
Model B: Focused	WTP €100,000 + Decreased Nivolumab	EUR100,000	EUR14730.07	u_np	EUR1251.48	8.5
Model B: Focused	WTP €150,000	EUR150,000	EUR12886.78	u_np	EUR488.12	3.8
Model B: Focused	WTP €150,000 + Decreased Nivolumab	EUR150,000	EUR21015.48	u_np	EUR590.73	2.8
Model C: Separate	Base Case	EUR 51,000	EUR153.69	u_np	EUR153.69	100.0
Model C: Separate	Decreased Nivolumab Cost	EUR 51,000	EUR4161.04	u_np	EUR232.83	5.6
Model C: Separate	WTP €100,000	EUR100,000	EUR3911.61	u_np	EUR399.62	10.2
Model C: Separate	WTP €100,000 + Decreased Nivolumab	EUR100,000	EUR15998.1	u_np	EUR1196.94	7.5
Model C: Separate	WTP €150,000	EUR150,000	EUR12556.28	u_np	EUR602.23	4.8
Model C: Separate	WTP €150,000 + Decreased Nivolumab	EUR150,000	EUR23778.67	u_np	EUR796.32	3.3

Table 17: Population-level EVPPI (EUR millions) for key parameters across scenarios and model structures. Based on 1500 patients/year over 10 years, discounted at 3.5%.

Scenario	Parameter	Model A: Joint	Model B: Focused	Model C: Separate
Base Case	u_np	2.40	2.00	1.92
Base Case	u_p	0.59	0.05	0.00
Base Case	c_drug_nivo	0.00	0.00	0.00
Decreased Nivolumab Cost	u_np	1.97	2.96	2.90
Decreased Nivolumab Cost	u_p	0.50	0.00	0.59
Decreased Nivolumab Cost	c_drug_nivo	0.17	0.21	0.24
WTP €100,000	u_np	4.85	4.02	4.99
WTP €100,000	u_p	1.07	0.04	0.00
WTP €100,000	c_drug_nivo	0.00	0.00	0.00
WTP €100,000 + Decreased Nivolumab	u_np	3.98	15.61	14.93
WTP €100,000 + Decreased Nivolumab	c_drug_nivo	3.76	10.54	3.25
WTP €100,000 + Decreased Nivolumab	u_p	1.06	5.40	1.57
WTP €150,000	u_np	7.35	6.09	7.51
WTP €150,000	u_p	1.56	0.03	0.00
WTP €150,000	c_drug_nivo	0.00	0.00	0.00
WTP €150,000 + Decreased Nivolumab	u_np	11.61	7.37	9.93
WTP €150,000 + Decreased Nivolumab	c_drug_nivo	5.07	2.33	2.98
WTP €150,000 + Decreased Nivolumab	u_p	1.07	0.62	3.32

10.3 Summary Comparison Table

10.4 Population-Level EVPPI Across Scenarios

10.5 Key Findings

10.5.1 Model Structure Robustness

Parameter importance is consistent across model structures. The parameter ‘u_np’ is the most important driver of value of information across all three model specifications. This suggests that conclusions about research priorities are robust to the choice of model structure.

10.5.2 Scenario with Greatest Decision Uncertainty

- **Model A: Joint:** WTP €150,000 + Decreased Nivolumab (EVPI = EUR22346.86)
- **Model B: Focused:** WTP €150,000 + Decreased Nivolumab (EVPI = EUR21015.48)
- **Model C: Separate:** WTP €150,000 + Decreased Nivolumab (EVPI = EUR23778.67)

10.5.3 Most Important Parameter Across All Scenarios

u_np (Total EVPPI = EUR9009.18)

10.5.4 General Observations

Impact of WTP Threshold: Higher WTP thresholds generally increase decision uncertainty (EVPI), creating greater value for additional research to reduce parameter uncertainty.

Impact of Nivolumab Cost: Decreased nivolumab costs shift the cost-effectiveness landscape and may change which parameters drive decision uncertainty.

Impact of Model Structure: Comparing EVPI and EVPPI across model structures helps identify whether value of information conclusions are robust to structural uncertainty in the survival model specification.

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Note on population-level EVPPI: Population-level values calculated assuming 1500 annual eligible MSS/pMMR mCRC patients in Norway (based on Cancer Registry data and international prevalence estimates), discounted at 3.5% per year over a 10-year research horizon.